DPP	DEPARTMENTAL POLICY AND PROCEDURE		
Title	**CARE OF NEWBORN WITH NECROTIZING ENTEROCOLITIS**		
Department	**Neonatology Department**		
Applies to	**All NICU Medical and Nursing Staff**		
Policy Code and Version Number	**DPP-NICU-010 (v2)**		
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APPROVAL DATE	EFFECTIVE DATE	TYPE OF REVISION	REVIEW DATE
19 / 3 / 2024	7 / 4 / 2024	TRIENNIAL	18 / 3 / 2027

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1.0 PURPOSE

1.1. To provide guidelines to health care providers on how to manage sick newborn with necrotizing enterocolitis.

1.2. To ensure that all medical and nursing staff are able to recognize NEC at an early stage to limit the progress of the illness and complications.

1.3. To emphasize the nursing care to be given to newborn with suspected and confirmed cases of NEC.

2.0 DEFINITION/ABBREVIATION

2.1. **NEC (Necrotizing enterocolitis)** is a serious bacterial infection in the intestine, primarily of sick or premature newborn infants that can cause the death (necrosis) of intestinal tissue and progress to blood poisoning (septicemia).

2.2. **PC02**- Partial Pressure of Carbon Dioxide

2.3. **CRP**-C-Reactive Protein

2.4. **FFP**- Fresh Frozen Plasma

2.5. **PRBC**-Packed Red Blood Cells

2.6. **PPE**- Personal Protective Equipment

2.7. **NICU**-Neonatal Intensive Care Unit

3.0 RESPONSIBILITY

3.1. **Physicians (Residents, Specialists, Consultant)** are responsible to assess patients for the presence of necrotizing enterocolitis.

3.1.1. The physician will provide a written order of the procedure to manage/ treat necrotizing enterocolitis.

3.2. **Nurse (Head Nurse, Charge Nurse, Staff Nurse)** are responsible in assessing and giving nursing care to newborn according to physician's ordered treatment for NEC.

3.2.1. The nurse will document any procedure done to patient in the medical file.

4.0 POLICY

4.1. All newborn with feeding intolerance with or without risk factors must have an early medical review and assessment in Nursery and will be subjected for admission to Neonatal Intensive Care Unit based on the newborn's clinical condition and physician's written order.

4.2. Assessment Criteria for Necrotizing Enterocolitis:

4.2.1. Abdominal Signs

4.2.1.1. Feeding Intolerance with increased naso/orogastric tube aspirate

4.2.1.2. Vomiting feeds, bile or blood

4.2.1.3. Abdominal distension, discoloration and tenderness

4.2.1.4. Abdominal wall erythema, abdominal mass

4.2.1.5. Decreased bowel sounds and blood in stools

4.2.2. Systemic Signs

4.2.2.1. Temperature Instability

4.2.2.2. Lethargy, Irritability

4.2.2.3. Apnea, Respiratory distress

4.2.2.4. Poor Capillary refill

4.2.2.5. Decreased urine output

4.2.2.6. Increasing metabolic acidosis

4.2.2.7. In ventilated infants a respiratory syndrome NEC consisting of decreased oxygenation and increased respiratory rate or increased PC02 may be seen.

4.3. Initial investigations such as full blood count and coagulation, Renal, Liver and Bone profiles, CRP, Blood culture, Stool culture and blood gas analysis must be done.

4.4. Abdominal X-ray and Abdominal Ultrasound must be initiated.

4.5. Additional ventilatory support, volume and inotropic support may be added depending on clinical condition.

4.6. Enteral feeds must be stopped with naso/orogastric tube free drainage with hourly aspiration.

4.7. Initiate Antibiotic Therapy:

4.7.1. Commence Cefotaxime, Vancomycin and Metronidazole.

4.8. Fluids and Nutrition:

4.8.1. For Resuscitation: Normal Saline bolus 10ml/kg to be repeated as required

4.8.2. Correct abnormal electrolytes and blood glucose

4.9. FFP, Platelets and PRBC transfusion will be administered to the newborn if indicated.

4.10. Minimal handling should be provided to the newborn patient with NEC.

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4.10.1. Confirmed NEC patients will be referred to higher facility if surgical intervention is indicated.

5.0 PROCEDURE (S)

5.1. The physician will assess the patient for necrotizing enterocolitis.

5.2. The physician will write the order of the procedure in the medical file according to his management.

5.3. The nurse will verify and carry out the physician's order.

5.4. Perform hand hygiene and wear appropriate PPE according to the procedure to be done.

5.5. Identify the patient correctly.

5.6. The nurse will proceed in performing the management for NEC (e.g. Fluid and Antibiotic Therapy etc.) according to physician's written order.

5.7. Provide the appropriate nursing care for patient with suspected/confirmed NEC according to physician's order and management.

5.8. Perform continuous re-assessment of the newborn patient.

5.9. Facilitate transfer of the patient if surgical intervention is indicated as per physician's order.

5.10. Document the procedure done to patient in the medical file.

6.0 EQUIPMENTS

6.1. PPE

7.0 ATTACHMENT/FORMS

7.1. Nurses Progress Notes (GDOH-NUR-NPN-203)
7.2. Physician Order Form (GDOH-INP-POF-079)
7.3. Patients Medical File

8.0 REFERENCES

8.1. http:/Users/Administrator/DownloadsNecrotizing-Enterocolitis-in-lnfants-on-NICU

8.2. medical-dictionary. thefreedictionary.com/necrotizing+enterocolitis

8.3. Saudi Central Board for Accreditation of Healthcare Institution (CBAHI) NICU standards, 3rd version, January 2016

9.0 REVISION CLAUSE

9.1. Modification of policy format, lay out and policy coding

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KINGDOM OF SAUDI ARABIA

وزارة الصحة
Ministry of Health

Hospital: _____ مستشفى:

Region: _____ المنطقة/المحافظة:

Dept./Unit: _____ القسم/الوحدة:

MRN: | | | | | | | | | | رقم الملف الطبي:

Name: _____ الاسم:

Nationality: _____ الجنسية:

Age: _____ ☐ سنة Years ☐ شهر Months ☐ يوم Days العمر:

Date of Birth: ____ / ____ / 14____ H ____ / ____ / 20____ تاريخ الميلاد:

Gender: ☐ Male ☐ Female الجنس:

PHYSICIAN ORDER FORM

ALLERGIC TO:

DATE	TIME	PHYSICIAN ORDERS	PHYSICIAN STAMP / SIGNATURE	RN INITIAL / JOB NUMBER

KINGDOM OF SAUDI ARABIA

وزارة الصحة
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Date of Birth: ___ / ___ / 14___ H ___ / ___ / 20___ تاريخ الميلاد:

Gender: ☐ Male ☐ Female الجنس:

NURSES PROGRESS NOTES FORM

DATE &TIME	NURSES NOTES (D-data A-action R-response)	REGISTERED NURSE INITIAL WITH ID NUMBER

Note: Write the time in each entry & affix your initial at the end of each paragraph. Document your complete Name, Initial, Job number, Date & Time at the closure of your documentation. Draw a line across empty spaces.

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